TAMAYUZ 10X

OPERATIONAL EXCELLENCE · WHAT THE RESEARCH SAYS

When hospitals standardized shift handoffs, the handoffs got better.

The results varied from study to study.

That is what four recent studies, published from 2023 to 2026, show.

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THE HANDOFF

What happens when a shift ends?

The doctor going off shift hands over what they know about each patient: how they are doing, which tests are pending, and what to do if things get worse.

Miscommunication at this moment is a leading source of medical errors. So resident doctors in 32 U.S. hospitals learned one standard way to hand over, with 18 months of coaching.

The method is called I-PASS. Its five steps: Illness severity, Patient summary, Action list, Situation awareness and contingency planning, and Synthesis by the receiver.

FINDINGS

Better handoffs, fewer reported adverse events

47%

drop in handoff-related adverse events, as reported by the residents themselves.

66%

of verbal handoffs covered all five key elements, up from 20%.

83%

of handoffs ended with the receiver summing up what they heard, up from 31%.

SYSTEMATIC REVIEW · 2025

Which handoff tool has the best evidence?

A systematic review looked at ten years of evidence on 12 handoff tools used within hospital units.

I-PASS reduces errors and adverse events, with moderate certainty, based on ten studies.

The older SBAR tool may improve safety, but with low certainty.

The other tools have not been tested enough to judge.

WHY DOES A STANDARD HELP?

The key information comes up every time, in the same order.

Then the receiver sums up what they heard, so a misunderstanding shows up before the other person leaves.

Patient safety then depends less on one person's memory at the end of a long shift.

This is the author's interpretation, not something the study measured.

A STANDARD IS NOT ALWAYS ENOUGH

Handoffs can improve while the main outcome stays the same

41%

fewer pressure injuries in a randomized trial on nine Australian wards, but the trial's main outcome did not change.

3/16

of the studies that measured adverse events found an improvement, in a review of 42 studies.

Results were more often positive when training came with changes to how the work is done.

WHAT THE STUDIES SUGGEST

What can leaders do?

1

Choose a tested handoff method, and ask the receiver to sum up what they heard.

2

Don't stop at training. Change the work: a standard template, a set time and place, and support in the first months.

3

Measure handoff quality by direct observation, and track outcomes over enough time.

> **Which handoff between two teams in your organization depends today on one person's memory?**

SOURCES AND LIMITS

What this evidence can and cannot tell us

The 32-hospital study had no control group, and adverse events were reported by the doctors themselves.

Some of its authors hold equity in a company that trains hospitals in I-PASS; an independent team ran the analysis.

None of the studies included Arab hospitals. Applying the idea to other sectors is an inference.

Editorial disclaimer: The views and analyses presented in this article represent the author's professional interpretation based on the cited sources and do not necessarily represent the official positions of the organizations mentioned. For the original, complete and most current information, please consult the primary sources and official documents listed in the references.

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